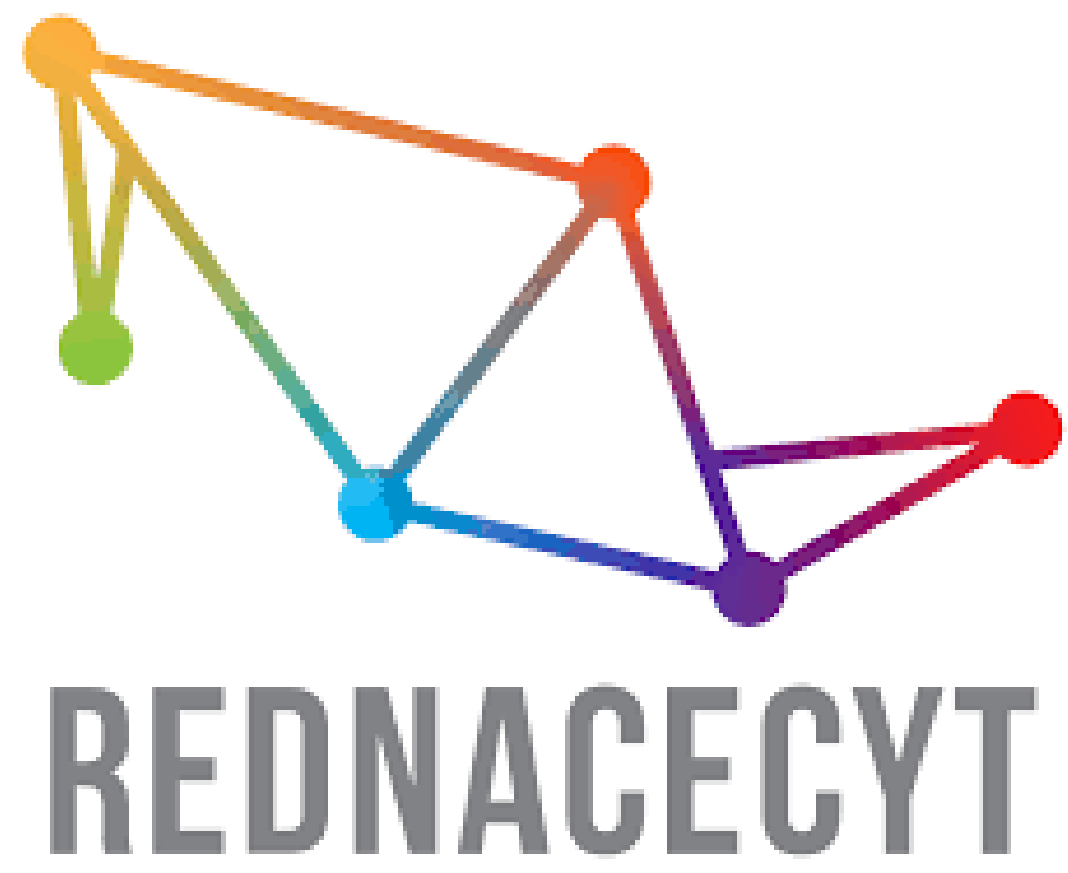




Traditional medicine in the health services of Amealco, through interculturality in health.

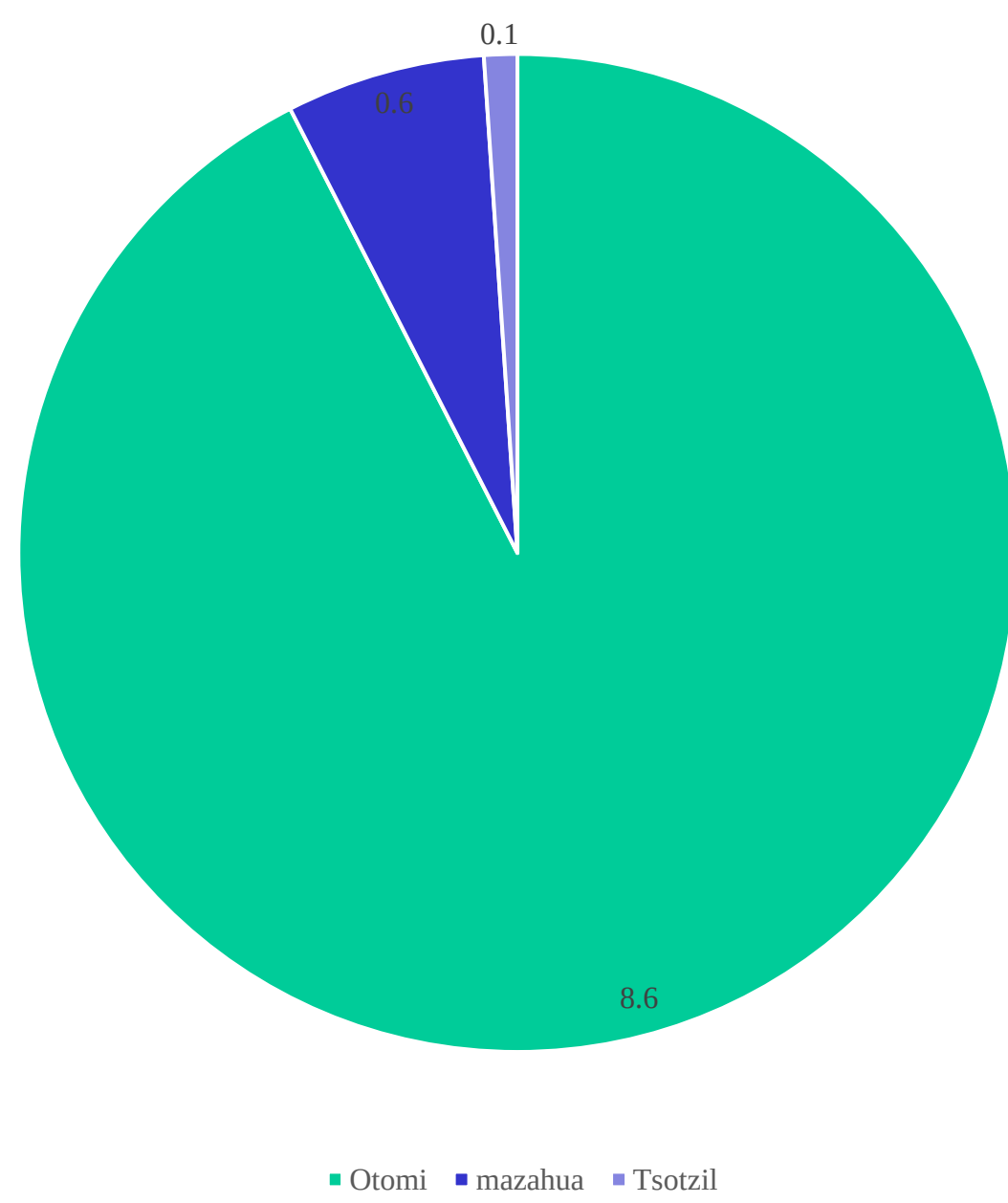
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INTRODUCTION

Intercultural policies in Mexico have served to characterize the coexistence in a space, of two models of medical care: the practices of traditional indigenous medicine and an established, hegemonic system of biomedical medicine. This is approached through respectful relationships and horizontality, with the objective of correcting health deficiencies in indigenous communities and giving recognition to traditional indigenous medicines.

Grupos indígenas



RESEARCH QUESTION

What are the ideologies and practices of traditional doctors in the face of the integration of traditional medicine into health services in a mixed hospital in Amealco?

GENERAL OBJECTIVE

To analyze the ideologies and practices of traditional doctors in the face of the integration of traditional medicine into health services in a mixed hospital in Amealco.

REFERENCES

BACKGROUND



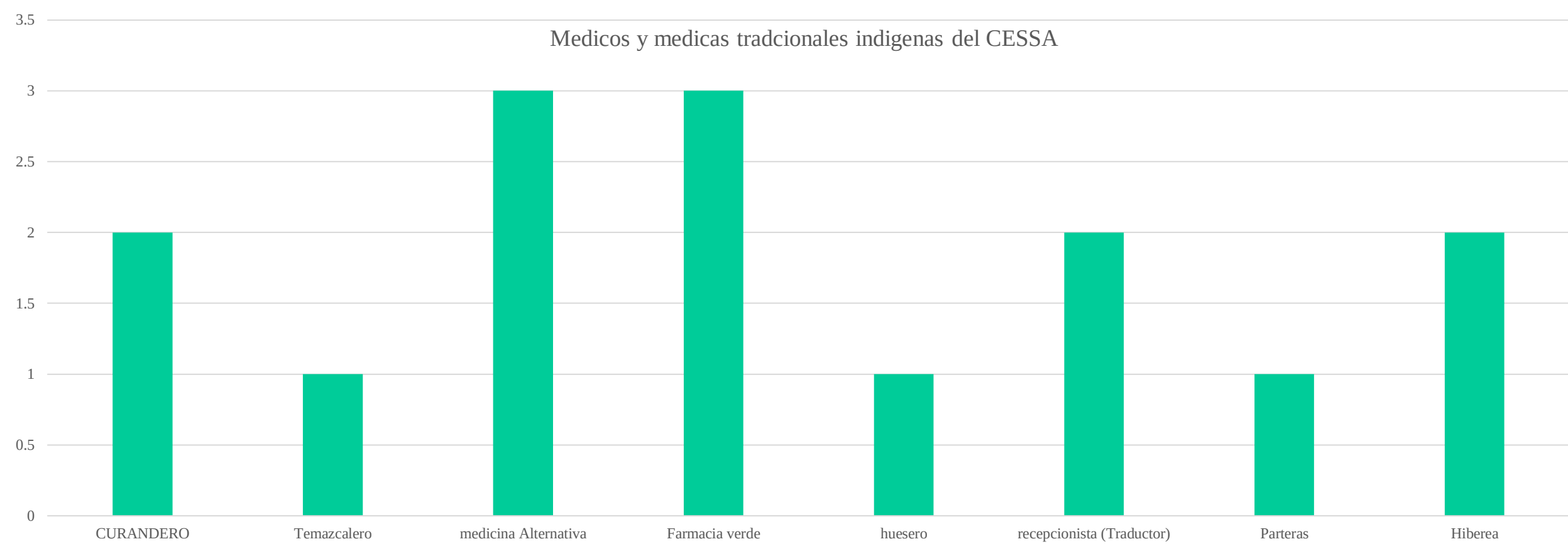
- In 1978 at the Primary Health Care (APS) conference, the need to include health personnel and traditional medicine practitioners to attend to health needs was considered.

The WHO issues the "Strategies on Traditional Medicine 2014-2023".



METHODOLOGY

- A qualitative research was conducted, with critical positioning, based on Van Dijk's critical discourse studies, using as a methodological technique for the production of information, 15 semi-structured interviews with traditional practitioners who have collaborated with health services in the Health Center with Extended Services (CESSA) of Amealco, an Otomí (Hñāhñü) indigenous community. Subsequently, the information was processed and analyzed based on the systematization of our main categories and subcategories, using Van Dijk's critical discourse analysis.



MAIN FINDINGS

- Interculturality in health maintains an integrationist perspective, which does not recognize the processes that determine health conditions in indigenous populations, since it is a strategy to bring people from the communities closer together.
- The knowledge of traditional indigenous medicine is transitioning to knowledge with more and more elements of biomedical medicine, which inclines it to be an alternative medicine and lose the connotation of indigenous.

MAIN FINDINGS

- Interculturality in health has served, not only to integrate traditional medicine practices into health services, but also alternative medicine practices and conceptions that are based on biomedical logic, no longer being a resource that is only practiced by people who belong to indigenous groups and communities.
- Interculturality in health highlights the role of traditional doctors in health services through situations of:
 - Subordination.
 - Submission.
 - Discrimination.
 - Racism.
 - Power.
- The use of medicinal plants and midwifery maintains a very important place for health on the immediate international stage, but from biomedical rigor, therefore, the cultural framework is being omitted.
- On the one hand, the ideologies of traditional doctors have evolved in two ways: one from the training that health institutions have used to be able to adopt them, thus losing key elements that are used in the communities. And, on the other hand, there is the syncretism that occurs once they come into contact with the institutions because they face situations of superiority and discrimination on the part of health personnel.

CONCLUSION

The selected recognition of certain practices in traditional medicine (herbalism, midwifery, massages) have served to incorporate them into a neoliberal market under the biomedical rigor where the cultural framework is omitted, acquiring more and more the character of alternative medicine.

The ideologies of traditional practioners have been influenced in two ways: one from the training that health institutions have used to be able to hire them and keep their activities as volunteers, thus losing key elements used in their communities. On the other hand, there is the syncretism that occurs once they come into contact with the institutions, as they face situations of superiority and discrimination by the health personnel, leaving aside certain beliefs and/or knowledge because they are discarded as backward, mainly by biomedical doctors, who through their actions determine and allow which traditional medicine practices should be performed.